CenterWell Home Health

PREPARED FOR
CWHH CLINICAL QUALITY TEAM

In their own words: what patients told us they want

Branch 0412 | CQI run 08/15/2026 | 2 high or rising-risk patients | Short summaries follow in this order

Patient 9000231 | Very high risk 0.55

No patient-stated goal recorded

What CQI found: 14 ordered visits remain; 5 scheduled; No vitals found for this episode.

Patient 9000123 | High risk 0.38

“I want to stay at home and live as long as I can.”

What CQI found: Weight up 6.3 lb in 3 weeks, possible fluid retention; 7 ordered visits remain and none are scheduled.

Patient-stated goals from the care plan, wording unchanged. AI-assisted findings are drafts for clinical review.

CenterWell Home Health

Vitals trajectory turns weekly readings into an action window

Branch 0412 | CQI run 08/15/2026 | patients ordered by urgency

Context + weekly vitals → Trajectory → Urgency + escalation → Action window

Patient 9000123

DETERIORATING

What CQI saw

Weight up 6.3 lb over 3 weeks; SpO2 95% to 91%; pulse crossed 100 on DAY+29; no readings since DAY+33.

What CQI flags

Possible fluid retention. Clinician review within 24 hours; no SN visit scheduled is inadequate monitoring.

CLINICIAN REVIEW WITHIN 24 HOURS

Patient 9000231

INSUFFICIENT DATA

What CQI saw

No vitals recorded for this episode.

What CQI flags

Monitoring gap: confirm the vitals plan with the SN.

OBTAIN VITALS AT THE NEXT VISIT

AI-assisted findings are drafts for clinical review.

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Clinical Quality Intelligence

Which patient needs care today, before it's too late?

Weekly Patient Conference Summary

High-Risk & Rising-Risk Patients | Week of 08/17/2026

CQI is a framework for identifying risk, care gaps and opportunities to improve patient outcomes. It is **proactive**: it shows which patient needs care this week, why, and what to do **while there is still time to change the outcome**.

Patient

Patient ID	**9000231**	60-day risk	**Very high 0.55**
Episode	**7000512**	Status	**Current, day 19 of 60**
Branch	**Branch 0412**	CQI run	**08/15/2026**

Produced by **CW Data Science & Applied AI - HH Team** Model **cwhh-tif-60day-v1**

Short summary. Full evidence and charts are in the full booklet.

At a glance

From the patient's stated goal to what needs attention now

From stated goal to what needs attention now

Patient goal	*No patient-stated goal recorded in the care plan*
Risk	Very high 60-day hospitalization risk (0.55); driven by dyspnea at rest, COPD primary diagnosis, home oxygen.
Care delivery	14 ordered visits remain; 5 scheduled; Shortfall to date: PT 1.
Clinical trajectory	No vitals recorded for this episode. Monitoring gap: confirm the vitals plan with the SN.
CQI flags	**Schedule remaining ordered visits (9 unscheduled).**

ACTION WINDOW	VITALS TRAJECTORY	DATA GAPS	DISCHARGE
Obtain vitals at the next visit	Insufficient data	0	Not due

Care network for discharge: No PCP on record | CW PCO: Consider | CW Rx: Already enrolled

CQI ANALYSIS

Vitals trajectory: action window

Weekly readings turned into a trajectory, an urgency and an action window

Patient 9000231 | Episode 7000512 | Branch 0412 | Day 19 of 60

WEEKLY VITALS
0 vitals days

TRAJECTORY
Insufficient data

URGENCY
No vitals

ACTION WINDOW
Obtain vitals at the next visit

What CQI saw

No vitals recorded for this episode.

What CQI flags

Monitoring gap: confirm the vitals plan with the SN.

VITALS DAYS	DAYS SINCE LAST READING	NEEDING REVIEW	TRAJECTORY
0	-	0	Insufficient data

What CQI found, by measure

Measure	First	Latest	Change	CQI interpretation	Urgency
-				No vitals findings	

AI-assisted findings are drafts for clinical review. Full evidence and charts are in the full booklet.

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Clinical Quality Intelligence

Which patient needs care today, before it's too late?

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Patient

Patient ID	**9000123**	60-day risk	**High 0.38**
Episode	**7000456**	Status	**Current, day 36 of 60**
Branch	**Branch 0412**	CQI run	**08/15/2026**

Produced by **CW Data Science & Applied AI - HH Team** Model **cwhh-tif-60day-v1**

Short summary. Full evidence and charts are in the full booklet.

At a glance

From the patient's stated goal to what needs attention now

From stated goal to what needs attention now

Patient goal	*“I want to stay at home and live as long as I can.”*
Risk	Readmission after HF hospitalization; AFib on anticoagulant, recent fall, lives alone with impaired vision.
Care delivery	Acuity window met. SN short in weeks 3 and 5, PT in weeks 4-5; OT evaluation and MSW consult not done.
Clinical trajectory	Weight up 6.3 lb over 3 weeks with new ankle edema; SpO2 drifting 95% to 91%. No vitals since DAY+33.

CQI flags

Call the patient and notify MD of the weight gain today. Schedule all remaining SN and PT visits and escalate to the clinical manager.

ACTION WINDOW	VITALS TRAJECTORY	DATA GAPS	DISCHARGE
Clinician review within 24 hours	Deteriorating	6	Not due

Care network for discharge: PCP on record | CW PCO: Refer | CW Rx: Refer

CQI ANALYSIS

Vitals trajectory: action window

Weekly readings turned into a trajectory, an urgency and an action window

Patient 9000123 | Episode 7000456 | Branch 0412 | Day 36 of 60

WEEKLY VITALS
10 vitals days, last DAY+33

TRAJECTORY
Deteriorating

URGENCY
Within 24 hrs

ACTION WINDOW
Clinician review within 24 hours

What CQI saw

Weight up 6.3 lb over 3 weeks; SpO2 95% to 91%; pulse crossed 100 on DAY+29; no readings since DAY+33.

What CQI flags

Possible fluid retention. Clinician review within 24 hours; no SN visit scheduled is inadequate monitoring.

VITALS DAYS	DAYS SINCE LAST READING	NEEDING REVIEW	TRAJECTORY
10	3	3	Deteriorating

What CQI found, by measure

Measure	First	Latest	Change	CQI interpretation	Urgency
SpO2 (%)	95	91	**-4 in 5 wks**	Gradual decline, still above limit; consistent with weight trend	**Within 24 hrs**
Weight (lb)	168	174.3	**+6.3 in 5 wks**	Steady gain with edema noted DAY+33; possible fluid retention	**Within 24 hrs**
Systolic BP (mmHg)	132	152	**+20 in 5 wks**	Rising but below limit; recheck with HF review	**This week**
Pulse (bpm)	82	98	**+16 in 5 wks**	Crossed 100 on DAY+29; AFib - confirm apical rate and rhythm	**Routine**

AI-assisted findings are drafts for clinical review. Full evidence and charts are in the full booklet.